MEMO

Summary

$180M in 2029 is not reachable based on the current evidence. Even with every assumption held true in the high case, the forecast predicts that the net revenue of $180M is really only achievable from 2030 onwards (hits $177M in 2029). However, in the low or base case, the highest the net revenue goes is ~$20M and ~$70M respectively. See the forecast and its range in one chart below (based off the numbers presented in Section 3: Ranges):

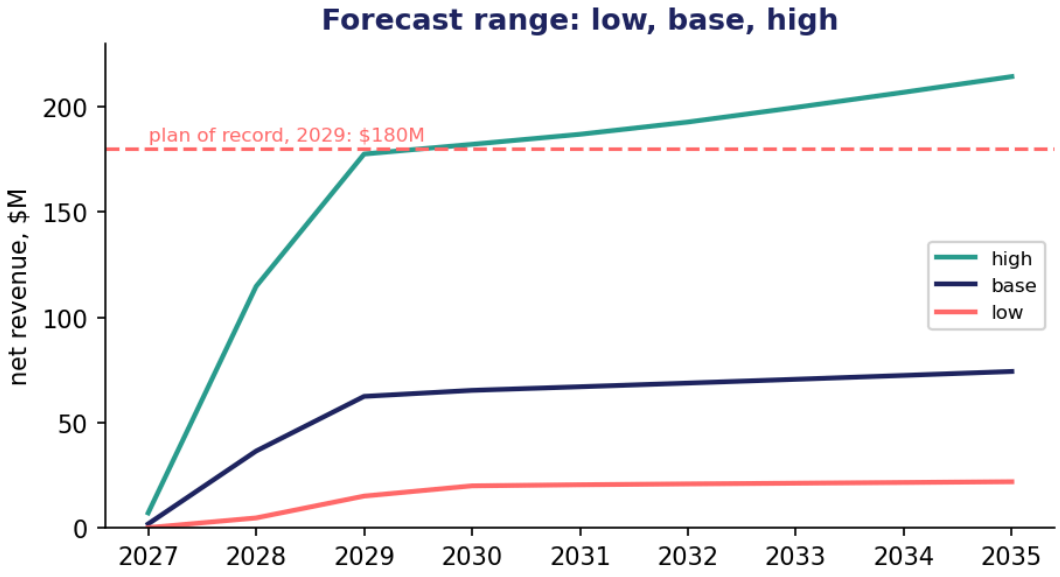


Figure 1. Net revenue in the low, base and high cases, 2027-2035, against the $180M plan of record.

Three key assumptions

Three key assumptions driving our forecast/ranges (from the Section 3 Tornado):

1. D1 Months on therapy (booked in start year)
2. B3 Peak share, 3L+
3. B4 Peak share, 2L

Given the above context, reaching $180M seems difficult unless we can substantiate that each of the three assumptions are going to held in the years following launch in 2027. In the next 90 days, researching into each of the 3 assumptions below-- duration-on-therapy data for D1 & PA/Payer/Competitor landscape for B3 and B4-- would help identify whether the plan of record is achievable.

Evidence that would change each assumption

1. We would need RWE on the time of treatment from EMR data, medical claims, trial data for Veltraxa, or even info on Rx refills. The more months here that can be proven, the better the revenue forecast.
2. For this, we would need to see prescription rates for patients who progressed on a BTK inhibitor or how competitors in 3L+ are doing as a relative proxy to adjust for. Stated use or interest will be strong, positive signals.
3. As the 4th drug in the BTK class, we would want to see the formulary positioning of Veltraxa or do some PA outreach (look into payer/prior-auth) to see if they would reccomend this over other established inhibitors. Requirements to try some class of this drug will account for the volume of treatment being pushed out.

Monthly budget and annual long-range plan

Below is the monthly budget and the annual long-range plan from the same model. For the monthly budget, new patients rise to ~13 in total, for roughly $2M of revenue in 2027. In the 2028, about 230 new patients adds $36.6M or ~$3M a month. The long range plan all the way through 2035 supports our conclusion above on the unlikeliness that the plan of record will be attainable.

- Phasing assumption: There's a linear ramp in the launch year (builds month by month from July - December 2027) and then it becomes flat afterwards (each month in 2028 gets the same amount). Flat phasing jumps revenue from $0.6M in December 2027 to $3.0M a month, which will obviously overstate the revenue from early in the year and then understate the revenue from later in teh year since the actual on-therapy count would build gradually.

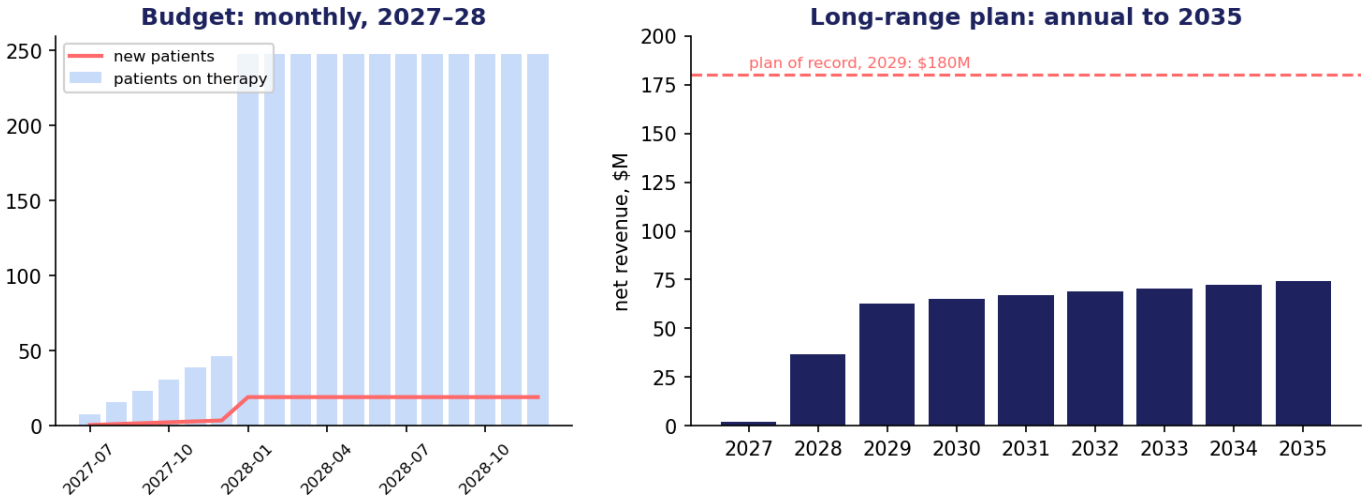


Figure 2. Monthly budget, Jul 2027-Dec 2028 (left), and annual long-range plan, base case, to 2035 (right).

Long-range plan, 2027-2035 (net revenue, $M)

Year	New patients (base)	Low	Base	High
2027	13	0	**2**	7
2028	230	5	**37**	115
2029	394	15	**62**	177
2030	412	20	**65**	182
2031	422	21	**67**	187
2032	433	21	**69**	193
2033	445	21	**71**	200
2034	456	22	**72**	207
2035	468	22	**74**	214

Monthly budget, base case

Month	New patients	Net revenue, $M
Jul 2027	0.6	0.1
Aug 2027	1.2	0.2
Sep 2027	1.8	0.3
Oct 2027	2.4	0.4
Nov 2027	3.0	0.5
Dec 2027	3.6	0.6
2027 total	**12.7**	**2.0**
Each month, Jan-Dec 2028	19.2	3.0
2028 total	**230.4**	**36.6**